SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Pembrolizumab + paclitaxel + carboplatin AUC6

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

First-line metastatic squamous NSCLC · NCCP 00579 · version 5a · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This combination aims to control metastatic squamous lung cancer, help symptoms and extend life. Your team weighs likely benefit, risks and what matters to you.

Schedule
Induction: all three medicines on Day 1 every 21 days for up to four cycles. Maintenance: paclitaxel and carboplatin stop; pembrolizumab alone may continue every 21 days while appropriate. The team agrees duration and review points.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Urgent symptoms and local contact details confirmed
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Support medicines and phase changes explained
- ☐ Common, serious and lasting drug-specific risks discussed below

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Pembrolizumab
IMMUNOTHERAPY

- ☐ Immune inflammation of lungs, bowel, liver, kidneys, glands, skin and other organs
- ☐ Serious infusion reactions
- ☐ Effects can develop after treatment ends

Paclitaxel
CHEMOTHERAPY

- ☐ Low blood counts and infection
- ☐ Hair loss
- ☐ Numbness or tingling that may persist
- ☐ Muscle/joint aches
- ☐ Infusion reactions

Carboplatin
CHEMOTHERAPY

- ☐ Low blood counts, infection or bleeding
- ☐ Nausea and vomiting
- ☐ Allergic reactions
- ☐ Kidney, hearing and nerve effects

Discuss expected benefit, alternatives and important short- and longer-term risks. Tell the team about all medicines, existing illnesses, pregnancy possibility and symptoms that matter to you.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00579 v5a.

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